

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 16/09/2026

Generata il: 2026-09-16

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **Resistenza / T315I / mutazioni** (n=5).
- Secondo filone: **Altro** (n=2).
- Terzo filone: **Sintesi (review/linee guida)** (n=2).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
 - Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
 - Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).
-

Top 5 da leggere subito (ordinati per impatto)

1) Ponatinib in Chronic Myeloid Leukaemia Refractory To Multiple Tyrosine Kinase Inhibitors: Data from a Tertiary Care Centre in India.

Real-world / osservazionale

Indian journal of hematology & blood transfusion :
an official journal of Indian Society of Hematology and Blood Transfusion Score: 13

Focus clinico: Resistenza / T315I / mutazioni, Safety e comorbidità

PMID: 42712753 | 10.1007/s12288-025-02184-8

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42712753/>

- **Tipo di studio/popolazione:** Allogeneic haematopoietic stem cell transplantation (HSCT) is currently indicated in Chronic Myeloid Leukemia (CML) patients who are intolerant or resistant to 2 or more Tyrosine Kinase Inhibitors (TKIs) or in patients with advanced stage disease.
- **Risultato chiave:** CML patients treated with ponatinib at our centre between 1st January 2019 to 28th February 2025 were assessed retrospectively.
- **Rilevanza clinica:** Out of the entire cohort, a total of 15 patients (41.66%) attained a Major Molecular Response (MMR) (RQPCR $\leq 0.1\%$) and 6 patients (16.66%) attained a Deep Molecular Response (DMR) (RQPCR $< 0.01\%$).

2) Efficacy and Safety of Ponatinib as a Second-Line Treatment for Chronic Phase Chronic Myeloid Leukemia: Retrospective Study at Two Japanese Centers.

Real-world / osservazionale

Indian journal of hematology & blood transfusion :
an official journal of Indian Society of Hematology and Blood Transfusion Score: 9

Focus clinico: Resistenza / T315I / mutazioni, TFR / stop terapia, Sequencing / linee di terapia

PMID: 42712751 | 10.1007/s12288-025-02301-7

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42712751/>

- **Tipo di studio/popolazione:** The third-generation tyrosine kinase inhibitor (TKI) ponatinib has demonstrated efficacy in patients with chronic phase chronic myeloid leukemia (CML-CP).
- **Risultato chiave:** This observational, multicenter, retrospective study included patients aged ≥ 16 years with CML-CP who received ponatinib as second-line therapy between September 2016 and October 2023 at Yokohama City University Medical Center and Fujisawa City Hospital.
- **Rilevanza clinica:** Five patients (31%) discontinued ponatinib.

3) Prescription Behaviour and Drug Interactions of Acid Suppressants With Tyrosine Kinase Inhibitors in Patients With Leukaemia: Data From Germany.

Linee guida / consenso

EJHaem

Score: 8

Focus clinico: Real-world (utilità pratica), Sintesi (review/linee guida)

PMID: 42713051 | 10.1002/jha2.70279

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42713051/>

- **Tipo di studio/popolazione:** Their absorption, however, is highly pH-dependent, making them susceptible to interactions with acid-suppressing agents such as proton pump inhibitors (PPIs), histamine-2 receptor antagonists (H2RAs) and antacids.
- **Risultato chiave:** Results indicate substantial co-prescription rates involving dasatinib, nilotinib, imatinib, bosutinib and ponatinib, with variations across years and agents.
- **Rilevanza clinica:** Pharmacokinetic evidence demonstrates that PPIs and H2RAs can significantly reduce systemic TKI exposure, with reported decreases of up to 96% in peak plasma concentration and 88% in overall exposure.

4) Allosteric kinase inhibition in hematological malignancies: from asciminib to emerging regulatory sites.

Review

Biochemical pharmacology

Score: 7

Focus clinico: Resistenza / T315I / mutazioni, Sintesi (review/linee guida)

PMID: 42702240 | 10.1016/j.bcp.2026.118436

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42702240/>

- **Tipo di studio/popolazione:** ATP-site resistance mutations, exemplified by T315I in BCR::ABL1, limit the durability of kinase inhibitor therapy in hematological malignancies.
- **Risultato chiave:** Several reviews have addressed this principle across the kinome, but none has applied a hematology-focused druggability appraisal anchored in the BCR::ABL1/asciminib precedent.
- **Rilevanza clinica:** In the ASCEMBL trial, it achieved a major molecular response rate of 25.5% at 24 weeks versus 13.2% for bosutinib in heavily pretreated CML, with better tolerability-the first regulatory-site inhibitor approved for a hematological malignancy.

5) Reframing chronic myeloid leukemia outcomes beyond survival: a perspective of quality of life and patient-centered data.

Trial clinico

Blood reviews

Score: 7

Focus clinico: Altro

PMID: 42701040 | 10.1016/j.blre.2026.101431

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42701040/>

- **Tipo di studio/popolazione:** The introduction and refinement of tyrosine kinase inhibitors (TKIs) over the past 25 years have resulted in dramatic improvements in life expectancy in individuals with chronic myeloid leukemia (CML).
- **Risultato chiave:** Patients with CML receiving TKIs have worse QoL than the general population, but TKIs have a generally less negative impact on QoL factors than previously available treatments for CML.
- **Rilevanza clinica:** The inclusion of patient-reported QoL outcomes in clinical trials will be vital to helping physicians understand the patient experience of CML and thereby improve disease management.

Evidenze per domande cliniche

Resistenza / T315I / mutazioni (5)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Ponatinib in Chronic Myeloid Leukaemia Refractory To Multiple Tyrosine Kinase Inhibitors: Data from a Tertiary Care Centre in India.

Real-world / osservazionale

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Open Access (Europe PMC/PMC): non rilevato

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Mini-riassunto (da abstract, estrattivo):

Efficacy and Safety of Ponatinib as a Second-Line Treatment for Chronic Phase Chronic Myeloid Leukemia: Retrospective Study at Two Japanese Centers.

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- **Tipo di studio/popolazione:** The third-generation tyrosine kinase inhibitor (TKI) ponatinib has demonstrated efficacy in patients with chronic phase chronic myeloid leukemia (CML-CP).
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- **Rilevanza clinica:** Five patients (31%) discontinued ponatinib.

Mini-riassunto (da abstract, estrattivo):

Allosteric kinase inhibition in hematological malignancies: from asciminib to emerging regulatory sites.

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Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Ponatinib plus asciminib in myeloid blast-phase CML harboring a phased <i>BCR</i>::<i>ABL1</i> Y253H/T315I compound mutation: a case report.

Case report

Frontiers in oncology

Score: 5

PMID: 42694182 | 10.3389/fonc.2026.1928961

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42694182/>

Open Access (Europe PMC/PMC): sì (PMC13537998)

- **Tipo di studio/popolazione:** Compound <i>BCR</i>::<i>ABL1</i> mutations, particularly those including T315I in combination with P-loop substitutions, represent a major therapeutic challenge in advanced-phase chronic myeloid leukemia (CML).
- **Risultato chiave:** We report a heavily pretreated patient with myeloid blast-phase CML in whom next-generation sequencing at blast transformation identified Y253H, T315I, and a low-level Y253_E255delinsHGK variant.
- **Rilevanza clinica:** Given the aggressive disease biology, limited therapeutic options, and transplant intent, the patient received ponatinib 45 mg once daily plus asciminib 200 mg twice daily as an individualized bridge strategy.
- **Metodi (full text OA):** non riportato.
- **Risultato principale (full text OA):** non riportato.
- **Dettaglio utile (full text OA):** non riportato.
- **Messaggio clinico (full text OA):** Asciminib 200 mg twice daily was selected to attempt intensive dual-site BCR::ABL1 inhibition in the context of a T315I-containing compound mutant, although the optimal dose of asciminib in combination with ponatinib in blast-phase CML is unknown.

Mini-riassunto (da abstract, estrattivo):

Revisiting Asciminib Resistance: From Binding Affinity to Allosteric Disruption in <i>BCR::ABL1</i> Leukemias.

Altro

Turkish journal of haematology : official journal of
Turkish Society of Haematology

Score: 4

PMID: 42706725 | 10.4274/tjh.galenos.2026.86383

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42706725/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** The first-in-class STAMP (Specifically Targeting the ABL Myristoyl Pocket) inhibitor, asciminib introduces a distinct therapeutic paradigm by targeting the myristoyl pocket and avoids direct competition with ATP.
- **Risultato chiave:** However, resistance to asciminib has been reported in 4-10% of chronic myeloid leukemia (CML) cases.
- **Rilevanza clinica:** Asciminib resistance is predominantly driven by impaired allosteric regulations: 1) Expanded spectrum of kinase domain mutations "pre-activate" the kinase and confer resistance despite retained drug affinity; 2) Intrinsic primary resistance, from b2(e13)a3 and b3(e14)a3 transcript variants; and 3) Isoform-specific (p190 vs.

Altro (2)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Reframing chronic myeloid leukemia outcomes beyond survival: a perspective of quality of life and patient-centered data.

Trial clinico

Blood reviews

Score: 7

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- **Risultato chiave:** Patients with CML receiving TKIs have worse QoL than the general population, but TKIs have a generally less negative impact on QoL factors than previously available treatments for CML.
- **Rilevanza clinica:** The inclusion of patient-reported QoL outcomes in clinical trials will be vital to helping physicians understand the patient experience of CML and thereby improve disease management.

Mini-riassunto (da abstract, estrattivo):

First line asciminib in chronic myeloid leukemia: a quest for the "Holy Grail" of tolerability.

Altro

Haematologica

Score: 3

PMID: 42719985 | 10.3324/haematol.2026.301753

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42719985/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** non riportato (abstract non disponibile).
- **Risultato chiave:** non riportato.
- **Rilevanza clinica:** non valutabile senza abstract.

Sintesi (review/linee guida) (2)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Prescription Behaviour and Drug Interactions of Acid Suppressants With Tyrosine Kinase Inhibitors in Patients With Leukaemia: Data From Germany.

Linee guida / consenso

EJHaem

Score: 8

PMID: 42713051 | 10.1002/jha2.70279

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42713051/>

Open Access (Europe PMC/PMC): sì (PMC13551348)

- **Tipo di studio/popolazione:** Their absorption, however, is highly pH-dependent, making them susceptible to interactions with acid-suppressing agents such as proton pump inhibitors (PPIs), histamine-2 receptor antagonists (H2RAs) and antacids.
- **Risultato chiave:** Results indicate substantial co-prescription rates involving dasatinib, nilotinib, imatinib, bosutinib and ponatinib, with variations across years and agents.
- **Rilevanza clinica:** Pharmacokinetic evidence demonstrates that PPIs and H2RAs can significantly reduce systemic TKI exposure, with reported decreases of up to 96% in peak plasma concentration and 88% in overall exposure.
- **Metodi (full text OA):** This study combines two approaches—(1) a systematic review summarizing the impact of acid suppressants on TKI pharmacokinetics and clinical outcomes, and (2) an analysis of German prescription data from the Insight Health co■prescription database, focusing on TKIs primarily used for CML.
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Sequencing / linee di terapia (2)

Score: 0

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Real-world / osservazionale

*Indian journal of hematology & blood transfusion :
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Dose strategy / consolidamento (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Cost-Effectiveness Analysis of Asciminib in Patients With Chronic Phase Chronic Myeloid Leukemia Previously Treated With 2 or More Tyrosine Kinase Inhibitors in Colombia.

Altro

Value in health regional issues

Score: 4

PMID: 42720643 | 10.1016/j.vhri.2026.101691

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42720643/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** To assess the cost-effectiveness of asciminib in adult patients with chronic-phase chronic myeloid leukemia who have been treated with 2 or more tyrosine kinase inhibitors, from a third-party payer perspective in Colombia.
- **Risultato chiave:** Asciminib was dominant compared with bosutinib and ponatinib, yielding more QALYs (0.41 and 0.20) and lower costs (-USD 47 334 and -USD 88 554).
- **Rilevanza clinica:** Asciminib was a dominant treatment option for patients with chronic-phase chronic myeloid leukemia who have been treated with 2 or more tyrosine kinase inhibitors, offering clinical and economic advantages over other pharmacological treatments.

Real-world (utilità pratica) (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

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Safety e comorbidità (1)

Score: 0

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TFR / stop terapia (1)

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